قائمة الأسئلة التدريبية

1. Which feature helps confirm a trigger point clinically?

a. Absence of tenderness
b. Local twitch response
c. Increased ROM
d. Bone deformity

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2. A trigger point is defined as:

a. A ligament tear
b. A nerve compression site
c. A bone deformity
d. A hyperirritable spot in a taut band of muscle

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3. Ischemic compression typically lasts:

a. 1-2 seconds
b. 5 seconds
c. 10 seconds to 2 minutes
d. 5 minutes

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4. The primary goal of myofascial release is to:

a. Enhance neural conduction
b. Stimulate bone growth
c. Break down fascial adhesions
d. Increase muscle strength

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5. Which receptor is MOST associated with relaxation during slow sustained MFR?

a. Muscle spindles
b. Ruffini endings
c. Free nerve endings
d. Pacinian corpuscles

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6. MFR works partly through pain reduction via:

a. Increased inflammation
b. Pain gate mechanism
c. Reducing blood flow
d. Blocking motor neurons

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7. The superficial fascia primarily functions to:

a. Increase joint stability
b. Facilitate movement of skin over underlying structures
c. Restrict skin movement
d. Prevent blood flow

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8. The phenomenon where fascia shows reduced resistance to a second stretch is called:

a. Elastic recoil
b. Plastic deformation
c. Hysteresis
d. Creep

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9. According to the Kinesio Taping method, how does the tape primarily assist with lymphatic drainage and pain reduction?

a. By producing a localized thermal effect that vasodilates superficial arteries.
b. By microscopically lifting the skin, forming convolutions, and increasing interstitial space.
c. By creating sustained tissue compression that manually forces fluid out of the limb.
d. By restricting the range of motion to prevent further tissue damage.

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10. Strain-counterstrain works by:

a. Increasing blood pressure
b. Stretching fascia aggressively
c. Increasing muscle spindle activity
d. Reducing muscle spindle input

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11. Which of the following correction techniques utilizes the lowest amount of available tension (0-15%) during application?

a. Ligament/Tendon Correction
b. Mechanical Correction
c. Space Correction
d. Lymphatic Correction

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12. Before applying Kinesio Tape, a physical therapist must screen the patient. Which of the following conditions is considered a contraindication for applying the tape?

a. Plantar fasciitis
b. Deep Vein Thrombosis (DVT)
c. Pre- and post-surgical edema
d. Carpal Tunnel Syndrome

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13. What is a primary physical characteristic of standard Kinesio Tape?

a. Constructed of synthetic, non-permeable fibers.
b. Bi-directional elasticity up to 200% stretch.
c. Unidirectional elasticity with 140% longitudinal stretch.
d. Contains a topical analgesic embedded in the adhesive.

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14. Referred pain in trigger points occurs due to:

a. Skin receptors only
b. Neural pathways linked to the muscle
c. Joint instability
d. Bone degeneration

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15. Which structure is MOST responsible for preventing tendon bowstringing?

a. Superficial fascia
b. Subserous fascia
c. Retinacula
d. Dermis

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16. Which of the following best describes fascia?

a. A structure found only in joints
b. Isolated connective tissue around muscles only
c. A continuous 3D connective tissue network throughout the body
d. A discontinuous structure limited to superficial layers

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17. When treating a patient for acute rehabilitation—specifically for an overused, over-contracted muscle with acute inflammation—what is the correct direction for tape application?

a. From the insertion of the muscle to the origin.
b. In a crisscross pattern directly over the joint line.
c. Horizontally across the muscle belly.
d. From the origin of the muscle down to the insertion.

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18. Satellite trigger points are:

a. Only found in ligaments
b. Primary pain sources
c. Secondary to a primary trigger point
d. Independent lesions

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19. Which of the following is a hallmark of an ACTIVE trigger point?

a. No pain
b. Only stiffness
c. Referred pain on palpation
d. No muscle involvement

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20. Which condition is a contraindication for MFR?

a. Chronic tightness
b. Fibromyalgia
c. Postural asymmetry
d. Open wound